National Cancer Control Programme (NCCP) 00742 · clinician draft

Lomustine + bevacizumab 5 mg/kg

Lomustine + bev 5 NCCP 00742: recurrent malignant glioblastoma; bevacizumab use is off-label. Use the exact prescription and schedule agreed with your specialist.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · lomustine and bevacizumab

Active medicines: Lomustine + Bevacizumab

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/dose	At least 30 minutes before the single Day1 lomustine dose; record delayed-emesis cover and rescue duration National Cancer Control Programme (NCCP) V6 Table5 p8 and timing p4. Review QT risk, liver function, constipation and interactions. Lomustine is a single dose, not a daily course.

Additional phase-specific support

- Lomustine has moderate-to-high oral emetic potential. Take the prescribed prevention and rescue plan around the single dose. Contact the team if nausea or vomiting prevents fluids or essential medicines; do not automatically repeat a vomited chemotherapy dose.
- Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.
- Keep prescribed blood-pressure medicine and urine checks. Follow the team's plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.
- Keep prescribed steroids and seizure medicines on their own schedules. Ask about missed or vomited doses. Agree a bowel plan if other medicines cause constipation and check interactions before adding a medicine.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00742 v3 · p5 · NCCP oral-systemic anticancer therapy (SACT) antiemetics V6: Table5 p8 and timing p4

Sources checked 2026-11-05 · SACTCheck v0.84.39 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Days 15 and 29 · bevacizumab only

Active medicines: Bevacizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Bevacizumab alone has minimal emetic potential. Routine anti-sickness drugs or steroid premedication are not automatically required for every infusion; the team may prescribe specific medicines after a previous reaction or for another reason.
- Keep prescribed blood-pressure medicine and urine checks. Follow the team's plan for any home measurements. Report wounds and upcoming surgery or dental treatment early, so timing and healing can be assessed before another infusion.
- Keep prescribed steroids and seizure medicines on their own schedules. Ask about missed or vomited doses. Agree a bowel plan if other medicines cause constipation and check interactions before adding a medicine.
- This is a bevacizumab-only visit. Do not repeat the Day 1 lomustine dose or automatically repeat its antiemetic course.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00742 v3 · p5 · Complete the individual local plan and interaction review before use; clinical/pharmacy approval pending

Sources checked 2026-11-05 · SACTCheck v0.84.39 · Regimen-level drafts; prescriber and oncology-pharmacy verification required